A/PROF CHAMARA BASNAYAKE

MBBS, PhD, FRACP

Gastroenterologist & Endoscopist

PN 421926PK

FOCUS GASTROENTEROLOGY

20 May 2026

Dr Demyana Ayoub
Saint Mary Medical Centre

Dear Dr Ayoub,

Re: Mrs Michele Traeger
DOB: 02/09/1958 Phone: 0422 508 404
Address: 1/70 McLaughlin Street, ARDEER, VIC, 3022
Email: micheletraeger17@gmail.com

Summary

- **Long segment Barrett's oesophagus; biopsies negative for dysplasia.**
- **Large hiatus hernia with associated Cameron erosions.**
- **Stable iron stores with no current requirement for supplementation.**
- **Surveillance gastroscopy recommended in two years.**
- **No action required by you.**

I had the pleasure of conducting a telehealth review with Michele following her gastroscopy. As you are aware, Michele has a known large hiatus hernia and long segment Barrett's oesophagus.

I am pleased to report that the extensive biopsies I performed within the Barrett's segment showed no evidence of dysplasia. Furthermore, biopsies from the stomach were negative for Helicobacter pylori or other concerning pathology, and duodenal biopsies were negative for celiac disease.

During the procedure, I noted Cameron erosions within the hiatus hernia. These erosions occur where the stomach rubs against the diaphragm and are a recognised cause of occult gastrointestinal blood loss and iron deficiency. Interestingly, Michele's iron stores are currently very good. This is a positive finding considering her last iron infusion was over a year ago and she is not currently taking any oral iron supplements.

Impression and Plan

Michele's reflux symptoms are currently well-controlled on her medication. She denies any significant heartburn or regurgitation. She has been taking her pantoprazole 40 mg at night after food; however, I have advised her that the medication is most effective when taken approximately 30 minutes before a meal. I have provided her with a 12-month authority prescription for pantoprazole 40 mg.

In light of the stable findings and the absence of dysplasia, I believe we can safely set her surveillance interval at two years. We will send a reminder when her next gastroscopy is due. We briefly discussed her surgical review; while she is currently managed medically, if she develops breakthrough symptoms or significant regurgitation, we may need to revisit the option of surgical repair for her hiatus hernia with Michael Hii

All appointments and enquires

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East Melbourne VIC 3002
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Email: office@focusgastro.com.au

Endoscopy locations

Epworth Freemasons Day Proc. Centre
320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065

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Yours sincerely,

A/Prof Chamara Basnayake
Gastroenterologist

CC: A/Prof Michael Hii

Investigation Results:

TRAEGER, MICHELE ANNE : MEH 569260, SVP 569260

DOB: 02/09/1958 Age: 67 years

Unit 1, 70 Mclaughlin St, Ardeer 3022

Results from 09/05/2026 15:15 Episode# 26P313383 Test Hist (Histology - Routine)

FINAL ANATOMICAL PATHOLOGY AAnat

Lab #: 2026B012205 **CLINICAL NOTES** Barrett's. **MACROSCOPIC DESCRIPTION** 1. Specimen labelled "28 cm oesophagus bx". Two mucosal fragments measuring 1-2 mm. A1. 2. Specimen labelled "30 cm oesophageal bx". Two mucosal fragments measuring 1-2 mm. A1. 3. Specimen labelled "Oesophageal bx 32 cm". Two mucosal fragments measuring 1-2 mm. A1. 4. Specimen labelled "34 cm oesophageal bx". Two mucosal fragments measuring 1-2 mm. A1. 5. Specimen labelled "36 cm oesophageal bx". Two mucosal fragments measuring 1-3 mm. A1. 6. Specimen labelled "Gastric bx". Two mucosal fragments measuring 2-4 mm. A1. 7. Specimen labelled "Duodenal". Three mucosal fragments measuring 1-2 mm. A1. **MICROSCOPIC DESCRIPTION** 1 to 5. Sections show tissue from the squamocolumnar junction. Intestinal metaplasia is noted, consistent with Barrett's oesophagus. Negative for dysplasia. Please see structured report below. 6. Sections show gastric antral type mucosa. No significant changes are seen. The lamina propria contains a normal complement of chronic inflammatory cells. No significant active inflammation is seen, and no Helicobacter organisms are identified in the routine stains. There is no evidence of intestinal metaplasia or dysplasia. 7. Sections of small bowel mucosa show no significant changes. The villous to crypt length ratio is within normal limits. The lamina propria contains a normal complement of chronic inflammatory cells, and there are normal numbers of intraepithelial lymphocytes. **PATHOLOGIST'S OPINION BARRETT'S OESOPHAGEAL BIOPSIES STRUCTURED REPORT 1. 28 cm Oesophageal bx Number of biopsies taken:** 2 **Epithelium present:** Squamous and columnar (with intestinal metaplasia) **Dysplasia:** Not identified. **2. 30 cm Oesophageal bx Number of biopsies taken:** 2 **Epithelium present:** Squamous and columnar (with intestinal metaplasia) **Dysplasia:** Not identified. **3. 32 cm Oesophageal bx Number of biopsies taken:** 2 **Epithelium present:** Squamous and columnar (with intestinal metaplasia) **Dysplasia:** Not identified. **4. 34 cm Oesophageal bx Number of biopsies taken:** 2 **Epithelium present:** Squamous and columnar (with intestinal metaplasia) **Dysplasia:** Not identified. **5. 36 cm oesophageal bx Number of biopsies taken:** 2 **Epithelium present:** Squamous and columnar (with intestinal metaplasia) **Dysplasia:** Not identified. **6. Gastric bx - No significant changes identified. 7. Duodenal bx - No significant changes identified.** Pathologist: Dr Matthew Egan

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